

ACTION PLAN

The 'Can't Sleep' Emergency Action Plan

Stop staring at the ceiling. Know exactly what to do when sleep won't come.

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What's Inside

- A step-by-step decision framework so you always know whether to stay in bed, get up, or switch strategies — no more frozen frustration
- 5 fast-acting, drug-free techniques proven to lower arousal and signal your nervous system that it is safe to sleep
- A clear rule for the exact moment you should get out of bed — and what to do when you do — so you stop making insomnia worse by accident
- Printable and phone-friendly format so your action plan is ready the second you need it at 3am

The 'Can't Sleep' Emergency Action Plan

START HERE: ANSWER 3 QUESTIONS

- Just... awake. No clear reason → Go to PATH E

Q3 — (Answer this on any path if it applies)

Did you check your phone, turn on lights, or watch TV to cope?

- Yes → Do the **Stimulus Reset** (bottom of page) before your path activity
- No → Go straight to your path

THE FOUR PATHS

■ PATH A — Can't Fall Asleep (Under 20 Min In Bed)

Stay in bed. Don't get up yet. Do this:

4-7-8 Breathing — 8 rounds (approx. 5 min)

- 1 Exhale completely through your mouth
- 2 Inhale through your nose for ****4 counts****
- 3 Hold for ****7 counts****
- 4 Exhale slowly through your mouth for ****8 counts****
- 5 Repeat 8 times without opening your eyes

Why it works: Extends exhale activates the parasympathetic nervous system, dropping heart rate and cortisol within minutes.

If you're still awake after this: Set a mental timer. When it feels like another 10 minutes have passed, move to Q2.

■ PATH B — RACING THOUGHTS

Get out of bed. Go to a dim room. Do this:

The Worry Offload — 5 minutes

You need: dim light, pen, paper (pre-set on a nearby surface before bed)

6

Write every thought currently in your head — no sentences required, just fragments

7

Next to each one, write ONE of these three words: *later / decide / drop*

8

Say quietly: **"I've noted it. My brain can let go."**

9

Leave the paper face-down

10

Return to bed

Why it works: Racing thoughts persist because the brain fears forgetting. Externalizing them onto paper satisfies that fear. The categorization step signals closure, not problem-solving — so you won't get energized.

What NOT to do: Don't start solving anything. If you catch yourself planning, write the word STOP and move on.

■ PATH C — PHYSICAL RESTLESSNESS / TENSION

Get out of bed. Stand or sit in dim light. Do this:

Progressive Muscle Release — 8 minutes

Work up the body. For each area: tense hard for 5 seconds, release, pause 10 seconds.

| Area | How to Tense |

|---|---|

| Feet | Curl toes under hard |

| Calves | Flex feet toward shins |

| Thighs | Squeeze legs together |

| Stomach | Pull navel to spine |

| Hands | Make tight fists |

| Shoulders | Shrug hard to ears |

| Face | Scrunch everything |

After the final release, stay still for 60 seconds. Notice the contrast between tension and release.

Why it works: Physical restlessness is often stored muscular tension that your nervous system is still processing. Intentional tension-release cycles discharge this faster than lying still hoping it passes.

Return to bed only when your body feels noticeably heavier.

■ PATH D — WOKE UP MID-NIGHT, CAN'T RETURN

The rules change here. Do this in order:

Step 1 — Do not look at the clock. Turn it away before you go to bed tonight. Looking at the time activates your problem-solving brain and creates math anxiety ("I only have 4 hours left").

Step 2 — Assess: Have you been awake over 20 minutes?

- If yes: get out of bed (see Step 3)
- If no: try PATH A breathing in bed

Step 3 — The Boring Audio Method — 10 minutes

- Go to a dim room
- Play something genuinely, deliberately boring at low volume: a documentary about something you don't care about, a slow podcast on an unfamiliar topic, an audiobook read in a monotone voice
- Lie on a couch or chair with eyes closed
- Do not try to sleep — just listen passively

Why it works: Middle-of-the-night waking is often tied to hyperarousal. Fighting it increases it. Passive, low-stimulation audio gives your alertness system something to occupy it while

your sleep drive quietly rebuilds.

When to return to bed: When you feel a wave of drowsiness — not before. Don't return hoping sleep will happen. Return when your body signals it's ready.

■ PATH E — JUST AWAKE (NO CLEAR REASON)

Get out of bed. Sit somewhere dim and still. Do this:

Cognitive Shuffle — 6 minutes

11

**Pick a random neutral word (not emotionally loaded). Example:
*CANDLE***

12

Visualize something that starts with each letter — one image per letter

- C → a ceramic cup
- A → an apple on a table
- N → a newspaper blowing in wind
- D → a dog sleeping
- L → a lamp in a corner
- E → an envelope

13

Make each image slightly random, unrelated, without narrative

14

If your mind wanders to real thoughts, gently return to the next letter

Why it works: Developed by cognitive scientist Luc Beaudoin, this technique mimics the semi-random imagery of natural sleep onset (hypnagogia), signaling to the brain that sleep is imminent. It's too random to maintain anxious focus.

Return to bed after the word is complete. Start a new word if you're still awake.

THE STIMULUS CONTROL RULE

THE MOST EVIDENCE-BACKED FIX IN SLEEP MEDICINE

The rule is simple:

> **Your bed is only for sleep and sex. Nothing else.**

This sounds obvious. It isn't. Every time you lie in bed awake — watching TV, scrolling, thinking, worrying, reading, eating — you train your nervous system to associate your bed with wakefulness. Eventually, getting into bed *triggers* alertness instead of sleep.

The fix:

- If you are awake in bed for more than 20 minutes: get up
- Do something calm and boring in dim light in another room
- Return only when sleepy — not just tired, actually sleepy (heavy eyes, drooping)
- Repeat every single time, every single night

This feels counterintuitive when you're exhausted. It works anyway. Studies show stimulus control therapy improves chronic insomnia faster and more durably than sleep medication, with effects that last long-term.

The short-term discomfort: A few nights of sleeping less as your brain re-learns the bed-sleep connection.

The long-term result: Getting into bed starts to feel effortlessly sleepy — the way it did when you were a child.

STIMULUS RESET

(Do this first if you've been on your phone or in bright light)

You've activated your alertness system. You need 5 minutes to reverse it before any path activity will work properly.

15 Put the phone face-down in another room

16 Turn off all overhead lights — one dim lamp only

17 Sit still with eyes closed for 2 minutes — no input of any kind

18 Take 5 slow breaths, making each exhale longer than the inhale

19 Now go to your path

Bright light and screens suppress melatonin and increase cortisol. Even 5 minutes of dim-only conditions begins to reverse this.

BEFORE BED: SET YOURSELF UP TO USE THIS

Place this page on your nightstand tonight.

Pre-set these nearby:

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Niche: Adults who lie awake at night unable to fall asleep and want a non-medication solution · Format: Action Plan ·
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